

Part 1: Understanding Medical Insurance Options for Seniors

Christa's Notes on Medicare, Medigap, Medicare Advantage Plans, and Medicaid Coverage

Medicare (aka "Medicare Part A", "Medicare Part B", and "Medicare Part D")

Medicare is insurance available to people 65+ that you pay for. "Original Medicare" consists of 2 parts:

- Part A (Hospital Insurance): Helps cover inpatient care in hospitals, skilled nursing facility care, hospice care, and home health care.
- Part B (Medical Insurance): Helps cover:
 - Services from doctors and other health care providers
 - Outpatient care
 - Home health care
 - Durable medical equipment (like wheelchairs, walkers, hospital beds, and other equipment)
 - Many preventive services (like screenings, shots, or vaccines, and yearly "Wellness" visits)

You can also join an optional drug coverage plan, called Part D, to help cover the cost of prescription drugs (including many recommended shots or vaccines).

You can see any provider anywhere in the US that accepts Medicare (most do), but there is typically a 20% co-insurance cost that you must pay each time you bill Medicare for a service. That can add up and there is no "out-of-pocket" max per year like you get with most "commercial insurers". (e.g., Aetna, Blue Cross Blue Shield, UnitedHealth, etc.)

Note: You can learn about Original Medicare starting with this link:

<https://www.medicare.gov/basics/get-started-with-medicare/medicare-basics>

Medigap Plans (aka "Medicare Supplemental Insurance")

Since Original Medicare only covers about 80% of most your annual medical costs. You are responsible for paying expenses for services. These "out of pocket" expenses can include:

- Deductibles – The amount you must pay for health care or prescriptions before Original Medicare begins to pay.
- Coinsurances – An amount you must pay as your share of the cost for services after your deductible is met. Coinsurance is usually a percentage (for example, 20%).
- Copayments – An amount you must pay as your share of the cost for a service or supply after your deductible is met. A copayment is usually a set amount (for example, \$10 or \$20).

Most people also get private supplemental insurance, known as Medigap insurance, to fill these gaps in coverage and save money on the out-of-pocket costs per service. While Medigap insurance is provided by commercial insurance companies, the Federal Center for Medicare and Medicaid Services (CMS) has defined "policies" around what is covered by these plans. There are 10 different plans, labeled A-N, to choose from. Each plan offers a different combination of benefits with specified out-of-pocket costs. You pay the commercial insurance company a monthly premium for your Medigap policy. You still pay

Part 1: Understanding Medical Insurance Options for Seniors

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Medicare for your Part B coverage. Insurance companies offering these plans must comply with the policies outlined by CMS but they can charge you whatever their hearts desire for their premiums so you must shop around. *The cost is lower for people who buy at a younger age and won't change as you get older.* Premiums may go up because of inflation and other factors, but not because of your age.

Your providers bill Medicare first and Medicare will pay its share of the Medicare-approved amount for covered health care costs. Then the bill gets sent to your Medigap insurance so they can pay their share.

In some states, you may be able to special type of Medigap policy called Medicare SELECT. These plans offer slightly lower premiums but limit your coverage to a very specific network of doctors and hospitals.

Notes: You can learn more about Medigap, including the difference between plans A-N, starting with this link: <https://www.medicare.gov/supplements-other-insurance/whats-medicare-supplement-insurance-medigap>

The "Supplements & other insurance" section of the Medicare.gov web site is a real eye-opener to read as there are all sorts of little caveats one should be aware of if counting on a Medigap policy to provide the care you need without going bankrupt.

Medicare Advantage Plans (aka "Part C Plans", "Bundled Plans", and "All-in-One Plans")

Medicare Advantage Plans are essentially another way to get your Medicare Part A and Part B coverage. These are Medicare-approved plans that are sold by commercial insurers who must follow rules set by Medicare. Per Federal guidelines, Medicare Advantage Plans must cover everything that Original Medicare Parts A and B cover. Most Medicare Advantage Plans include drug coverage (Part D) and extras like vision and dental. Unlike Original Medicare, Medicare Advantage Plans have a yearly limit on what you pay out of pocket for services.

With Advantage Plans, you still pay Medicare monthly for your Part B coverage but you pay your commercial insurer a monthly fee for all your other coverage like Part A, Part D, vision, dental, etc. All your claims go through your commercial insurance and your providers are paid by your commercial insurance, versus by Medicare directly. Also, your commercial insurer also gets paid a fixed amount by Medicare for your care each month.

There are several types of Medicare Advantage Plans, each with their own special rules about **how** you get your Medicare covered Part A and B services and any supplemental benefits. Examples of these 'how you get service rules' include whether you need a referral to see a specialist and whether the provider you see must be in the insurer's approved network. Medicare Advantage Plan types include:

- Health Maintenance Organization (HMO) Plans
- Preferred Provider Organization (PPO) Plans
- Private Fee-for-Service (PFFS) Plans
- Special Needs Plans (SNPs)
- Medical Savings Account (MSA) Plans

Part 1: Understanding Medical Insurance Options for Seniors

Christa's Notes on Medicare, Medigap, Medicare Advantage Plans, and Medicaid Coverage

In addition to the different rules for each type of plan, each Medicare Advantage Plan insurer can charge you very different out-of-pocket costs and different monthly premium costs. These costs, along with the rules about how you get your care, can change each year. For these reasons, the actual “advantages” of Medicare Advantage Plans over Original Medicare can be significant or non-existent. You really need to shop carefully.

At the time of this writing, the general rule of thumb is to avoid Medicare Advantage Plans if you have serious medical conditions. Instead, go with Original Medicare plus a Medigap plan approach. However, there are certain Medicare Advantage Plans that offer incredibly low out-of-pocket maximums and a wide variety of in-network providers. These truly advantageous Medicare Advantage Plans are mostly available to retirees from large companies or members of powerful labor unions who have worked with a commercial insurer to develop plans with highly favorable rules and costs.

Note: For more detailed information, this is a great resource on what you can expect with a Medicare Advantage Plan: <https://www.medicare.gov/Pubs/pdf/12026-Understanding-Medicare-Advantage-Plans.pdf>

Medicaid

Medicaid (M-CAID) is a joint federal and state program that provides health coverage for some people with limited income and resources. While the Federal government sets specific program guidelines, each state is given the flexibility to run their M-CAID program as they see fit within those standards. Most states have several different M-CAID programs that target different audiences. For example, separate programs for low-income families, pregnant women, disabled individuals, elderly, etc. Each of these programs will offer different benefits to the participant, as well as have different functional/clinical and financial eligibility criteria.

M-CAID will cover nursing home care and custodial care services provided you meet very specific clinical and financial criteria. This is often referred to as Long Term Care Medicaid, LTC Medicaid, or Institutional Medicaid. Medicaid also offers Home and Community Based Services (HCBS), which are programs that provide services to individuals who live outside of nursing homes. Individuals enrolled in a HCBS program reside at home or in their community. The terminology, “Community,” is meant as a distinction between someone who does not live in their own home, but does not live in a nursing home. So, community can refer to the home of a caregiver, a family member, a board and care home, an assisted living residence, adult foster care home, or a senior living community.

M-CAID Waivers are state specific Medicaid programs that allow for services to be provided outside of nursing homes. HCBS programs are generally a type of Medicaid waiver program. Waiver programs allow states to by-pass certain Federal rules/guidelines for care. Waiver programs really vary from state to state. Some states provide long-term care to seniors through HCBS programs, some through their standard state Medicaid program, and some through BOTH types of programs. This can lead to a lot of confusion about long-term care coverage. Just because you're eligible to participate in a Waiver program does not mean that you will receive assistance. Waiver programs are generally NOT entitlement programs; There are a limited number of participant slots and waitlists may exist. Vermont

Part 1: Understanding Medical Insurance Options for Seniors

Christa's Notes on Medicare, Medigap, Medicare Advantage Plans, and Medicaid Coverage

has invested a lot of pushing their Choices for Care program, which is a M-CAID waiver program designed specifically to help those needing care to avoid nursing homes.

For seniors, here are some examples of care services often covered by M-CAID but not M-CARE:

- Adult Day Care / Adult Day Health
- Personal Care Assistance (at home, adult foster care homes, and assisted living facilities)
- Medical / Non-Medical Transportation
- Respite Care (to give the primary caregiver a break)
- Congregate Meals / Meal Delivery
- Home Health Aide / Skilled Nursing
- Home Modifications (widening of doorways, installation of ramps, etc.)
- Personal Emergency Response Systems
- Housekeeping / Chore Services
- Companion Services Transition Services (from nursing home back to home)
- Therapies (physical, occupational, and speech)
- Medication Administration

Both M-CAID and M-CARE provide Durable Medical Equipment, such as wheelchairs and walkers.

Qualifying for M-CAID

Unlike M-CARE, which is administered/run by the Federal government, M-CAID is state-administered and each state has different rules about eligibility and applying for M-CAID. In general, you need to have very limited income and assets to be eligible for M-CAID.

In specific circumstances, like when you need benefits not covered by M-CARE, you can have both M-CAID and M-CARE coverage. M-CAID never pays first for services covered by M-CARE. M-CAID only pays after M-CARE, Medigap, or Medicare Advantage Plan insurances have paid.

Note: The American Council on Aging has great information about planning for M-CAID for long-term care here: <https://www.medicaidplanningassistance.org/medicaid-eligibility/#>

In the state of Vermont, a senior citizen qualifies for M-CAID funded care in a M-CAID approved nursing home when their income is under \$2,523 / month and their assets are less than \$2,000. Typically, a person receiving M-CAID services must use all their monthly income to pay for their care before M-CAID kicks in. In Vermont, a person receiving M-CAID funded care in a nursing home can keep \$72.66 per month for their personal allowance. The income and asset limits vary for married couples vary depending on whether one or both parties are applying for M-CAID.

Regarding the \$2,000 asset limit, in most cases, one's home, home furnishings, and vehicle are exempt from being factored into a person's *eligibility* for M-CAID.* If you think you can just transfer your assets to your children and still qualify for M-CAID, you are mistaken. Asset transfers you make up to five years preceding your application date are scrutinized. This is referred to as the Medicaid Look-Back Period. If

Part 1: Understanding Medical Insurance Options for Seniors

Christa's Notes on Medicare, Medigap, Medicare Advantage Plans, and Medicaid Coverage

you've given away or sold assets for less than market value, this will count against your eligibility and you'll need to figure out a way to come up with the cash.

***IMPORTANT:** Be careful here, just because while your home is exempt for being counted against you for qualifying for M-CAID eligibility, doesn't mean it's safe from M-CAID cost recovery. If there's no surviving spouse, M-CAID will try to recoup the cost of your long-term care from the sale of any of your assets. The M-CAID Estate Recover Program (MERP) program is mandatory in all states following the death of anyone who received M-CAID funded care in a nursing home, at home, or in the community. Some states only seek estate recovery through assets that go through probate (e.g., Vermont), while other states use an expanded definition of "estate" and seek reimbursement through assets that do not go through probate (e.g., New Hampshire).

Note: Learn more at about MERP at <https://www.medicaidplanningassistance.org/can-medicaid-take-my-home/>